

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Fexofenadine and/or Dymista for the treatment of Allergic Rhinitis

Patient Group Direction, version 002, issued 10 September 2026. Supersedes Version 001, 25 June 2025.

Summary of NICE CKS Guidelines for Allergic Rhinitis

Overview

- **Allergic rhinitis** is an **IgE-mediated inflammatory condition** of the nasal mucosa triggered by allergens (e.g. pollen, dust mites, animal dander).
- Can be **seasonal** (e.g. hay fever), **perennial**, or **occupational**.
- Often associated with **asthma**, **eczema**, and **conjunctivitis**.

Symptoms

- Sneezing
- Nasal congestion
- Rhinorrhoea (runny nose)
- Nasal itching
- Postnasal drip
- Itchy/watery eyes (if allergic conjunctivitis also present)

Assessment

- Diagnosis is clinical based on symptom pattern and triggers.
- Determine if:
 - **Intermittent** (<4 days/week or <4 weeks) or **persistent** (≥4 days/week and ≥4 weeks)

- **Mild** (normal sleep/daily activities) or **moderate/severe** (disturbed sleep/impacts activities)

Consider:

- Trigger history (seasonal vs year-round)
- Coexisting asthma or eczema
- Impact on quality of life

Management

1. Allergen Avoidance and Education

- Limit exposure to known triggers (e.g. close windows, use pollen filters).
- Saline nasal irrigation can be helpful.

2. Pharmacological Treatment (Stepwise Approach)

Step	Treatment	Notes
First-line	Non-sedating oral antihistamines (e.g. loratadine, cetirizine)	Good for sneezing, runny nose, itchy eyes
Add or switch to	Intranasal corticosteroid spray (e.g. fluticasone, beclometasone)	Most effective monotherapy for nasal symptoms
Additional options	- Antihistamine eye drops (e.g. olopatadine) for ocular symptoms	
	• Intranasal antihistamines (e.g. azelastine) • Decongestants (short-term only, max 7 days) Combine with nasal steroids if needed • Consider combination sprays (e.g. fluticasone + azelastine) for moderate/severe symptoms.	

3. Immunotherapy (Specialist Only)

- Consider for confirmed **IgE-mediated allergy** (e.g. pollen) with **persistent symptoms** not controlled by medications.
- Administered **subcutaneously or sublingually**.

When to Refer

- Poor response to optimal treatment
- Diagnostic uncertainty
- Suspected structural abnormalities (e.g. nasal polyps)
- Consider referral for **allergy testing** or **immunotherapy assessment**

Monitoring and Follow-Up

- Review symptom control and medication adherence.
- Assess for complications: sinusitis, sleep disturbances, or asthma flare-ups.
- Adjust treatment based on response and seasonal variation.

Patient Group Direction

for the supply/administration of

Fexofenadine 180mg

for the treatment of

Allergic Rhinitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Fexofenadine is indicated for the symptomatic relief of allergic rhinitis, including seasonal hay fever, in adults and adolescents aged 12 years and over.
Inclusion criteria	- Individuals aged 12 years and over with symptoms consistent with allergic rhinitis. - Informed consent obtained. - No contraindications to fexofenadine. - Previous diagnosis of allergic rhinitis or recurrence of known symptoms.
Exclusion criteria	- Known hypersensitivity to fexofenadine or any component of the formulation. - Severe renal or hepatic impairment. - Pregnancy or breastfeeding. - Under 12 years of age.
Cautions	- Caution in individuals with a history of cardiovascular disease. - Advise avoidance of alcohol and other sedating antihistamines. - If symptoms persist beyond 7 days or worsen, refer to a healthcare provider. - Ensure patients are aware this is a non-sedating antihistamine but occasional drowsiness may still occur.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Fexofenadine 180mg
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration with water before food.
Dose and frequency	180 mg once daily.
Quantity to be administered and/or supplied	Up to 30 tablets (1 month supply).
Maximum or minimum treatment period	As required for symptom control, typically seasonal use.
Adverse effects	Headache, drowsiness, dizziness, nausea. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and
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	inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/08/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/08/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Dymista

for the treatment of

Allergic Rhinitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Dymista is indicated for the relief of moderate to severe seasonal or perennial allergic rhinitis in adults and adolescents aged 12 years and over, where monotherapy with either intranasal antihistamine or corticosteroid is not sufficient.
Inclusion criteria	- Adults and adolescents aged 12 years and over. - Symptoms of moderate to severe allergic rhinitis requiring dual therapy. - Informed consent obtained. - No contraindications to Dymista or its components.
Exclusion criteria	- Known hypersensitivity to azelastine, fluticasone, or any excipients. - Untreated fungal, bacterial, or viral nasal infection. - Recent nasal surgery or trauma. - Children under 12 years of age.
Cautions	- Advise on correct nasal spray technique to optimise efficacy and reduce side effects. - Caution in individuals with hepatic impairment, glaucoma, or tuberculosis. - Reassess need for continued treatment if symptoms persist beyond 4 weeks. - Counsel on possible side effects and need for ongoing review if used long-term.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Dymista
Legal category	POM
Storage	Store below 25°C. Do not refrigerate or freeze.
Route/method of administration	Intranasal spray, one spray in each nostril twice daily.
Dose and frequency	One spray in each nostril twice daily (morning and evening).
Quantity to be administered and/or supplied	One bottle (23 g), approx. 120 sprays.
Maximum or minimum treatment period	Use as required; assess effectiveness after 2–4 weeks.
Adverse effects	Nasal irritation, headache, epistaxis (nosebleeds), bitter taste, somnolence. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/08/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/08/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Version and change record

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 25 June 2025

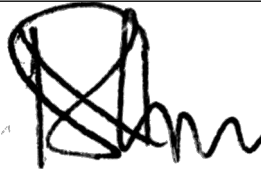
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The previous version carried three different dates for one version (issued 25 June 2025, signed 25 August 2025, valid from 25 August 2026) and empty validity cells.

Signed on behalf of Get Real Health

Version v002, 10 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		10 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		10 September 2026

Both authorising signatories reviewed this version together on 10 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.